



File No.: EXP202405180

RESOLUTION OF RIGHTS PROCEDURE

Considering the complaint registered on March 6, 2024, with this Agency and the procedural actions provided for in Title VIII of Organic Law 3/2018, of December 5, on the Protection of Personal Data and Guarantee of Digital Rights (hereinafter, LOPDGDD), the following FACTS have been established:

FIRST: On March 6, 2024, a complaint was received by this Agency submitted by A.A.A. (hereinafter, the complainant) against CLÍNICA ANABEL REPULLO, S.L. (hereinafter, the respondent) for not having duly addressed their right of access to their medical history.

The complainant states that, by means of a burofax dated February 12, 2024, they requested access to their personal data and a complete copy of their medical history, receiving a burofax from the respondent dated February 27, 2024, in which they were informed that all the documents requested were available to them at the Court of First Instance and Instruction ***COURT.1, where they are represented as a private prosecution. However, in the aforementioned complaint, they continue to state the following:

"The right of access is incomplete, as is the Medical History. While it is true that part of the history is in the court, numerous documents specified in the content of the burofax sent on February 12 of this year to the clinic responsible for the treatments are missing. For example, of the three declarations of conformity corresponding to the three prostheses that were placed on me: one has not been provided (piece No. 15, although some documents state No. 14), another is incomplete (pieces from 11 to 22), and the third (pieces from 24 to 27), I was given a declaration of conformity dated 4/12/17 when the prosthesis was placed on 7/03/19 (15 months later!!). In the same medical history, it is noted (according to the annotations of 21/12/18 from the clinic itself) that a new prosthesis is being made (two prostheses cannot have the same declaration of conformity!!). Another example: of the seven types of treatments I have undergone, I only have three informed consents; I do not have the informed consents for the following treatments: EXTRACTIONS (No. 14, 11, 12, 21, 22, and 26), GINGIVECTOMY, BONE GRAFT, and DENTAL WHITENING. Neither half of the budgets (not even signed), nor the itemized invoices (I do not have any), etc... This clinic also does not address the request for the data and information required in the form that the AEPD makes available to citizens for the EXERCISE OF THE RIGHT OF ACCESS, which was attached in the aforementioned burofax."

Along with the complaint, the following is provided:

- Burofax dated February 12, 2024, sent by the complainant to the respondent, with two documents:
 - o Exercise of the right of access to their personal data.

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Written document stating that the claimant has received medical treatment at the healthcare center of the defendant and "(...) I request that a complete copy of my medical history be provided to me, including medical notes, progress sheets, all tests performed, and informed consent documents for each of the treatments.

Additionally, I request all estimates and detailed invoices for the prosthetic and dental treatments I have undergone, as well as the documentary file for each of the prostheses that are part of the treatment (medical prescription, manufacturing guide, dates of tests and delivery, declarations of conformity, etc...).

I also request that I be informed of the name and registration number of the healthcare professionals who have participated in my treatment. Furthermore, I request that a copy of the professional liability insurance policy (including general and specific conditions) covering this incident be provided to me, both from the clinic and from the doctors who have participated in my treatment (...)."

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Burofax dated 26/02/2024 sent by the defendant to the claimant indicating that “We inform you that all the documents you requested in your burofax dated 12/02/2024 are at your complete disposal at the Court of First Instance and Instruction ***COURT.1, where you are represented as a private prosecution, so you can obtain the copies you need of said documentation, part of which has been provided by you, however, we remain at your disposal to provide you with any part of your medical history or documents to which you have a right of access that you specify.”

SECOND: In accordance with Article 65.4 of the LOPDGDD, this claim was forwarded to the defendant for analysis and to inform this Agency within one month of the actions taken to comply with the requirements set forth in data protection regulations. The notification of this transfer occurred on April 12, 2024.

On May 6, 2024, the defendant stated in response to this transfer procedure, in summary, that once the request for access to the claimant's medical history dated February 12, 2024 was received, they sent a burofax, dated February 26, 2024, responding that “(...) all the data you request - your medical history - is deposited in the Court of First Instance and Instruction No. ***COURT.1 (...)

We also indicated in our response that she, (...) is represented in the referenced case as a private prosecution, so the documentation you request, Copy of your Medical History, referring to Article 18 of Law 41/2002, of November 14, regulating Patient Autonomy, was already provided to her at that time. We proceed to detail the origin of this situation; on 15/03/2019 we received notification from the Official College of Dentists of Seville, requesting, on behalf of the patient, a complete copy of all data contained in her medical history for the treatment performed at our clinic. This documentation was duly provided, as recorded in a handwritten note and stamp from the College, dated...

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26/03/2019. On 28/03/2029, we received at the clinic a response from the College acknowledging receipt of the complete medical history of the patient deposited by the clinic, as requested. We provide for your verification the corresponding documents that substantiate this (Annexes 3 and 4).

After the delivery of a copy of the Medical History, as requested by the claimant, no request for access to the parts of her medical history that she considers missing has been received at the clinic from her. Had she done so, and in the event of any omissions on our part, we would have considered how to facilitate her right of access to what she indicated.

However, we proceeded to contact the claimant through a burofax dated April 30, 2024, again making available to her a complete copy of her Medical History, as evidenced by (Annex 5).

Following the transfer by the AEPD of the notification of this complaint, we consider that the copy of the Medical History provided to the claimant at the time is complete, including all documentation concerning said Medical History, and therefore it is correct, unless we are informed otherwise, and it is not necessary to provide a copy of it here as evidence. (...)

The incident that originated the complaint has been, according to the claimant, A.A.A., the incomplete and inadequate attention to her right of access to the medical history.

While it is true that, once the complaint was studied by the responding party, it seems possible that the background of this complaint is due more to discrepancies between the parties and the alleged dissatisfaction of the patient with the medical practice employed by the clinic in the treatments carried out, rather than to any problem in the attention to that right.

We believe that the clinic has provided all the required documentation, and therefore we understand that her right of access has been addressed and responded to, in accordance with data protection regulations. However, we express our full willingness to continue providing her with anything or clarification regarding her Medical History that she deems appropriate, and these data remain available to judges and competent public administrations in the event of a possible filing of a lawsuit.”

The following documentation has been provided:

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Annexes 1 and 2 are those already submitted by the claimant along with the complaint letter.
Annex 3: Letter dated 15/03/2019 from the Official College of Dentists of Seville, "(...), we have received the attached letter from the patient Ms. (...), therefore we request that we claim a complete copy of all the data contained in her medical history for treatment at her Dental Clinic Anabel Repullo. Therefore, we request that within a period of (...) you send us a complete copy of the patient's medical history (including tests such as p.j.):

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Informed consents, models, CT scans, photographs, X-rays, ...) to respond to your request"
Attached is a copy of the "Authorization for the request of medical history" signed by the claimant dated 14/03/2019 and stamped upon entry at this College on the same date.

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Annex 4: Document dated 28/03/2019 from the Official College of Dentists of Seville, "(...) we acknowledge receipt of the complete medical history deposited by Clínica Dental Anabel Repullo (including evidence such as informed consents, models, photographs, X-rays...), we proceed to forward it to the patient in response to her request."

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Annex 5: Burofax sent, with acceptance by Correos dated 30/04/2024 from the responding party addressed to the claiming party.

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Document dated 29/04/2024 in which they indicate that, following the complaint filed with the AEPD, "(...) we proceed to resend you a copy of your entire medical history. Additionally, we send the additional information regarding the processing of your data, including information on the exercise of rights."

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Document "Additional information on data protection" which states:

- o who is responsible for the processing of your data,
- o for what purpose we process your data,
- o how long we will retain your data,
- o what is the legal basis for the processing of your data,
- o to whom your data will be communicated,
- o what your rights are when you provide us with your data.

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Annex 6: "Declaration of conformity Date of sending 04/12/2017" from the Dental Laboratory Cromass "As the manufacturer, we declare that the medical product described below meets the essential requirements established in (...)

Medical prescription: against as Patient the claiming party and as Client the responding party.

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Annex 7: "Declaration of conformity Date of sending 25/06/2018" from the Dental Laboratory Cromass "As the manufacturer, we declare that the medical product described below meets the essential requirements established in (...)

Medical prescription: against as Patient the claiming party and as Client the responding party.

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Annex 8: "Declaration of conformity Date of sending 25/10/2018" from the Dental Laboratory Froilan S.C. "As the manufacturer, we declare that the medical product described below meets the essential requirements established in (...)

Medical prescription: against as Patient the claiming party and as Client the responding party.

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Annex 9: "Informed consent for general dentistry" Contains the details of the claiming party (but the Document appears undated and unsigned, difficult to read).

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Annex 10: "Informed consent for the performance of fixed prosthesis" Document dated 15/02/2017, signed by the claiming party.

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Annex 11: "Informed consent for surgery related to implantology" Document dated 11/01/2018, signed by the claiming party.

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For the reasons stated, this Agency, in its resolution dated May 17, 2024, considered that the right had been addressed, having been notified to the claimant on May 17, 2024.

THIRD: Dissatisfied with the resolution, the claimant filed a motion for reconsideration arguing that a complete response had not been provided to the data requested in the right of access.

The claimant explains that the declarations of conformity must meet a series of requirements, as established in Royal Decree 1591/2009, of October 16, which regulates medical devices, among others: the name of the doctor or authorized person who made the corresponding prescription and, where applicable, the name of the healthcare center, and the specific characteristics of the product indicated in the prescription:

"(...) The declaration that the product in question complies with the essential requirements set out in Annex I of this royal decree and, where applicable, the indication of the essential requirements that have not been fully met, stating the reasons."

* Note: When this document refers to the numbers of dental pieces, it is according to the FDI dental numbering system.

- Regarding the declaration of conformity corresponding to the prosthesis of dental pieces numbered 12 to 22 (page 57 of the clinical history), the name and registration number of the prescribing doctor of the prosthesis are not included (the name of the doctor must appear in the declaration of conformity and not be replaced by that of the center, as in the aforementioned annex it states "...and, where applicable..." and not "...or, where applicable..."). The specific characteristics of the product indicated in the prescription (which dental pieces it corresponds to, manufacturing materials, etc.) are also not included.

- Regarding the declaration of conformity corresponding to the prosthesis of piece number 14 (page 58 of the clinical history), it is fortunate that it has appeared, as I have been requesting it for more than 5 years (since March 2019), even judicially, and it was not provided to me. Like the previous one, it does not include the name or registration number of the prescribing doctor of the prosthesis. The nomenclature and/or numbering of the prosthesis does not correspond to that of the piece it replaces. The specific characteristics of the product indicated in the prescription (which dental pieces it corresponds to, manufacturing materials, etc.) are also not included.

- Regarding the declaration of conformity corresponding to the prosthesis of pieces numbered 24 to 27 (page 59 of the clinical history). As can be observed in the declarations of conformity themselves, all include an identification number. Specifically, this one (No. 181003003, dated 04/12/17) belongs to the prosthesis placed on 04/12/2017. On 05/12/17, while removing said prosthesis, the ceramic coating broke (I have this prosthesis in my possession and it is not placed in the mouth). A new prosthesis is made (notes from the clinical history dated 02/10/18 and 08/10/18), which is placed definitively 15 months later (on 07/03/2019 according to the notes from the clinical history). I understand that the declaration of conformity dated 04/12/2017 belongs to the first prosthesis made and not to the second. Two prostheses cannot have the same declaration of conformity.

Therefore, I understand that the three declarations of conformity are not valid.

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Second: Regarding informed consents, Law 41/2002, of November 14, the basic regulatory law of patient autonomy and rights and obligations in the field of information and clinical documentation, states in its Article 8:

Article 8.

1. (...)

2. Consent shall generally be verbal.

However, it shall be provided in writing in the following cases: surgical intervention, invasive diagnostic and therapeutic procedures, and, in general, the application of procedures that involve risks or inconveniences of notable and foreseeable negative repercussions on the patient's health.

3. The written consent of the patient shall be necessary for each of the actions specified in the previous point of this article, reserving the possibility of incorporating annexes and other general data, and it shall have sufficient information about the application procedure and its risks...

In the signed informed consent for General Dentistry, the treatments of dental whitening and bone grafting are not explained, nor are the risks, possible side effects, or possible consequences in case these treatments fail. It is also not recorded in the clinical history that it has been communicated verbally.

Therefore, such informed consents are lacking.

Third: Regarding the invoices, in Law 29/2006, of July 26, on guarantees and rational use of medications and health products, the thirteenth additional provision states:

"The placement or delivery of custom medical products by a healthcare professional, in the exercise of their professional duties, shall not be considered dispensing, marketing, sale, distribution, supply, or placing on the market of the same, for the purposes of Articles 4.1 and 111. In any case, the healthcare professional must separate their fees from the manufacturing costs."

In the invoices listed on pages 51 to 54 and page 56 of the clinical history, the fees of the healthcare professional have not been itemized from the costs of the prostheses.

On the other hand, the invoice corresponding to the treatment of tooth number 14, which includes the extraction of the existing tooth, placement of an implant (page 9, note dated 27/02/18), and placement of a prosthesis (page 10, note dated 04/10/18) is not found in the history.

Therefore, I understand that the invoices, apart from not all being recorded in the clinical history (treatment of tooth no. 14, FDI dental nomenclature system), are incorrect.

Fourth: Regarding the budgets, the budget corresponding to the actions carried out on dental piece number 14 (extraction of dental piece number 14, bone graft, placement of implant, and placement of implant-supported crown) is missing.

Fifth: If we consult the SPANISH CODE OF DENTAL ETHICS AND DEONTOLOGY, prepared by the General Council of Dentists (Professional Organization of Dentists of Spain, which encompasses the Dental Colleges throughout Spain) through its Central Ethics Committee; in its Article No. 3 Scope of application, section 1 states:

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"1- The provisions of this Code are binding on all dentists in the practice of their profession, regardless of the modality in which they practice and irrespective of their social, religious, political ideology, or any other condition that may interfere with the quality of their professional performance." ...

We continue reading, and in Article 8 "Identification of the practitioner" it states:

"Teamwork shall not prevent the patient from knowing which professional assumes responsibility for their care and who is responsible for providing the necessary information, without prejudice to the additional information that must be provided by the professional performing the intervention."

From the Spanish CODE OF ETHICS AND DENTAL DEONTOLOGY, it is extracted that I have the right to be provided with the names and registration numbers of the professionals responsible for each and every treatment I have undergone at the clinic Anabel Repullo Anaya, S.L.

Sixth: The clinic to which the complaint is directed cannot claim that the documents requested for access have not been specified, as the writings sent via burofax dated 12/02/2024 clearly specify which documents access is requested for, and from which I received (via burofax dated 29/02/2024) the following response:

“... the requested documents are located in the Court of First Instance and Instruction ***COURT.1,....”
“... however, we remain at your disposal to provide any HC part or document to which you are entitled that you specify.”

In light of the response received from the clinic, I proceed to file a complaint with the AEPD dated 06/03/24. It is following this complaint that the clinic sends me (via burofax with notification date 03/05/24) a copy of the medical history, of which I provide a copy with poor quality, which is a copy of the digitized file deposited at the post office. (...) I take the liberty of attaching a copy of the medical history that was provided to us by the clinic, at the time (March 2019), through the Official College of Dentists of Seville (CODS).

Therefore, and by virtue of the above, it is REQUESTED:

(...)- The declarations of conformity (corresponding to each prosthesis and in accordance with the established regulations) for the prosthesis belonging to teeth no. 12 to no. 22, for the prosthesis belonging to tooth no. 14, and for the prosthesis belonging to teeth no. 24 to no. 27.

- The Informed Consents for the dental treatments not specified in the already signed Informed Consents, pertaining to the bone graft performed on 27/02/2018 (page 9 of the HC) and to the dental whitening performed on 24/10/2018 (page 10 of the Medical History).

- The itemized invoices, in accordance with the regulations, for the prosthetic treatments pertaining to teeth no. 12 to no. 22, no. 14, and no. 24 to no. 27.

I request that the clinic provide me with the names and registration numbers of the professionals responsible for the treatments, specifying which dentist is responsible for each treatment. (...)

Along with the written document, various documents related to the treatments received, as well as invoices issued for their execution, are provided.

On August 21, 2024, the appeal filed was sent to the responding party within the framework established in Article 118.1 of Law 39/2015, of October 1, of the

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Common Administrative Procedure of Public Administrations (hereinafter, LPACAP) for the purpose of allowing the submission of allegations and the presentation of documents and evidence deemed appropriate, without a response having been given.

The resolution of the aforementioned appeal for reconsideration indicates that “In this regard, the responding party, by not responding to the hearing procedure of this appeal for reconsideration, has not demonstrated compliance with the right of access to the medical history requested, therefore the appeal filed is to be upheld.”

FOURTH: As a consequence of the acceptance of the aforementioned Appeal for Reconsideration, the present Rights Procedure was initiated, transferring the documentation to the responding party on September 24, 2024, so that within ten working days it could present the allegations it deemed appropriate, stating in summary that:

“FIRST. - It should be noted that this party was not notified of the appeal for reconsideration filed by A.A.A. against the resolution of the AEPD dated 05/17/2024, which was admitted. Had this appeal been notified to us, the corresponding allegations would have been made.

SECOND. - Clínica Repullo SL has provided A.A.A. with all the documents, both medical and commercial, as well as all the data and diagnostic evidence in the possession of the Clinic, arising from the treatment to which she was subjected.

This claim should be dismissed, as the requests of the claimant have been duly attended to, in absolutely everything that was possible within the law.

I make the clarification about what was legally possible because it is evident that we must not alter

documents or create them ex profeso, under penalty of committing forgery, which seems to be what A.A.A. is proposing to us. Let me explain.

The documents in question are what they are and have been completed as the professionals in charge deemed appropriate at the time.

The fact that a specific declaration of conformity for dental prosthetics, whose issuance does not correspond to the clinic but to the dental prosthetics laboratory, meets or does not meet the requirements set forth in Annex VIII of RD. 1591/2009 of October 16, is a matter that must be resolved by the competent authorities in the field, which are the health authorities. What we cannot do in any way is alter the documents issued by the manufacturers.

Likewise, it is not permissible to alter or modify invoices that have already been issued and accounted for.

The fact that the invoices do not include the breakdown required by D.A. 12^a of RDL 1/2015 of July 24 (not D.A. 13^a of Law 29/2006 as erroneously invoked by the claimant) is a matter that will correspond, if applicable, to the health or consumer authorities.

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Likewise, if a particular action was not documented in the budget at the time, it is not permissible to subsequently create a budget document and backdate it.

Regarding informed consents, the issue borders on the absurd, as there are only two options: they were given or they were not; but if they were not given – as the claimant unfoundedly asserts – we do not understand how they can now claim them, once the treatment, for which they allegedly did not provide informed consent, has concluded. In other words, the claimant seeks to obtain a signed document acknowledging that they never signed it.

However, if what they intend to report is that the informed consents, considering the specific intervention they refer to, are defective, we reiterate that it is not permissible to alter the document in question now, as the health authorities are the competent bodies to assess its suitability.

We reiterate that all documentation generated has been provided to A.A.A., and we add that this delivery has been made even though there was no obligation to do so, for the reason I will now explain.

It has already been mentioned that all the documentation requested by A.A.A. was available to them in the Preliminary Proceedings ***REFERENCE.1 of the Court of First Instance and Instruction ***COURT.1, in which the claimant is present as the accusing party.

These Preliminary Proceedings are ongoing against the appearing party, who, like any citizen investigated in a criminal case, has the right to remain silent and not testify, which includes the right not to be compelled to provide documentation related to the investigated facts.

It seems that the claimant intends to violate my rights as an investigated party, compelling me to provide, at the headquarters of this Data Protection Agency, those documents that I would have the right not to be obliged to present in the Preliminary Proceedings ongoing against me in the Court of Instruction.

In any case, we have submitted to the College of Dentists of Seville, in judicial proceedings and before this Agency, all the documentation in our possession, without relying at any time on the right not to do so, as we have nothing to hide.

This Agency has verified that the transfer of the appeal filed by the claimant was sent to the respondent on August 21, 2024. Furthermore, the file contains the "notification rejection due to expiration" issued by the Unique Enabled Electronic Address or DEHú, which indicates that the notification "(...) sent by the Spanish Agency for Data Protection on 21/08/2024 at 11:40 has expired due to expiration, as the established deadline for appearance has been exceeded, as of 01/09/2024 at 00:00."

FIFTH: After examining the document submitted by the respondent, it is forwarded to the claimant, so that, within ten working days, they may submit their allegations.

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that it considers appropriate, stating, in summary, and as it pertains to the present procedure, that: "(...) The aim is, on one hand, to obtain documentation that, while it is not the clinic's responsibility to prepare, we understand it is the clinic's responsibility to ensure it reaches the patient in accordance with regulatory requirements. On the other hand, to demonstrate before this agency that the documentation that cannot be provided is due to the fact that throughout the treatment, the clinic has not fulfilled its obligations to the patient, as outlined in current legislation, such as Law 41/2002, of November 14, which regulates the autonomy of the patient and rights and obligations regarding information and clinical documentation, Royal Decree 1591/2009, of October 16, which regulates medical devices, Royal Legislative Decree 1/2015, of July 24, which approves the consolidated text of the Law on guarantees and rational use of medications and medical products, etc. Additionally, it fails to comply with the Spanish Code of Ethics and Dental Deontology itself, as we still do not know the name and registration number of the responsible professional for each and every treatment (something mandated by the Code) to which the claimant has been subjected at the clinic Anabel Repullo Anaya, S.L., understanding that the clinic is violating the claimant's right to information.

SECOND.- We completely disagree with the statement in the written allegations of the respondent, dated October 3, 2024: "The documents in question are what they are and are filled out as the professionals in charge deemed appropriate at the time," as the documents are not as they should be, not due to misfortune, nor due to a meteorological phenomenon, nor due to an accident. The reason that various documentation is incomplete, erroneous, or defective, and nonexistent (in some cases) is that the individuals working at that clinic did not perform their work professionally (from the medical aspect to the administrative aspect, including management), as a medical professional performs their work in accordance with the requirements of the regulations, legislation, and their own deontological code, and not as one deems appropriate, thereby failing to comply with medical, administrative, and patient information guarantees.

THIRD.- Regarding the declarations of conformity, although it is not the clinic's responsibility to prepare them, it must ensure that the manufacturer provides such declarations as required by legislation. This clinic has also not requested from the prosthetist the declarations of conformity as required by the regulations (having the obligation to keep the declaration of conformity available to the competent authorities for a period of five years from when the prosthesis is placed in the patient's mouth). And this is not about a specific declaration of conformity for a dental prosthesis; it concerns all of them. For example, one of them with number 20180723113106 (belonging to number 14, which coincidentally, and coinciding with this claim, has appeared after five years upon request) is absurd: it does not include the name or the registration number of the prescribing professional for the prosthesis. The nomenclature and/or numbering of the prosthesis does not correspond with that of the piece it replaces. The specific characteristics of the product indicated in the prescription do not appear, etc. We remind once again of Royal Decree 1591/2009, of October 16, ANNEX VIII:

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2. The declaration shall include the following indications:

2.1 For custom-made products:

The name and address of the manufacturer.

Data that allows for the identification of the product in question.

The statement that the product is intended to be used exclusively by a specific patient, and the name of that patient, an acronym, or a numerical code.

The name of the prescribing physician or authorized person who made the corresponding prescription

and, where applicable, the name of the healthcare center.

The specific characteristics of the product indicated in the prescription.

Therefore, based on the documentation provided by the clinic and the requirements set forth in the legislation, we have identified the following deficiencies:

1. Declaration of conformity 181003003:

- does not contain the details of the prescribing physician of the prosthesis.
- does not contain the specific characteristics of the product.

2. Declaration of conformity 20180723113106:

- does not contain the name of the prescribing physician of the prosthesis.

3. Declaration of conformity 2180109184245:

- does not correspond to the prosthesis that is placed in the mouth, as the prosthesis to which the aforementioned declaration of conformity corresponds (placed on 04/12/2017) was replaced by a new prosthesis (placed on 07/03/2019, dates according to the notes in the clinical history). Therefore, we do not have the declaration of conformity for the prosthesis numbered 24 to 26.

The declarations of conformity are documents that contain all the information about the prosthesis. We understand that, being incomplete (and nonexistent in one case), the clinic again violates the claimant's right to information.

FOURTH.- Regarding the informed consents, we do not understand what is absurd about requesting the informed consents corresponding to all treatments received at this clinic. The purpose of this request is to demonstrate that the clinic has not provided the claimant with all the necessary informed consents, because if it had done so, they would be properly signed and the clinic could have submitted them as part of the clinical history. It is worth recalling that Law 41/2002, of November 14, which regulates the autonomy of the patient and rights and obligations in terms of information and clinical documentation, states: (...)

The medical informed consent is not merely an administrative document that gives permission to the professional to carry out a specific treatment, as could be the acceptance of a budget to repair, for example, a vehicle. The informed consent is a document that contains vital information and is part of every patient's right to receive adequate information about a medical procedure in order to freely decide whether to undergo it or not. Therefore, we understand that the clinic and its professionals have again violated the claimant's right to information.

FIFTH.- This last additional provision is not incorrectly invoked, as it remains in force since it is not repealed by deriving from a higher-ranking law, and does not oppose what is established in Royal Legislative Decree 1/2015, which approves the consolidated text of the Law on guarantees and rational use of medications.

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and medical devices, as this Royal Legislative Decree incorporates the modifications made in the matter since the entry into force of Law 29/2006, of July 26.

- Law 29/2006, of July 26, on guarantees and the rational use of medicines and medical devices.

Additional provision thirteenth.

The placement or delivery of custom medical devices by a healthcare professional, in the exercise of their professional duties, shall not be considered as dispensing, marketing, sale, distribution, supply, or placing on the market of the same, for the purposes of articles 3.1 and 101. In any case, the healthcare professional must separate their fees from the manufacturing costs.

- Royal Legislative Decree 1/2015, of July 24, approving the consolidated text of the Law on guarantees and the rational use of medicines and medical devices.

Additional provision twelfth. Placement or delivery of custom medical devices by a healthcare professional.

The placement or delivery of custom medical devices by a healthcare professional, in the exercise of their professional duties, shall not be considered as dispensing, marketing, sale, distribution, supply, or

placing on the market of the same, for the purposes of articles 4.1 and 111. In any case, the healthcare professional must separate their fees from the manufacturing costs.

As can be seen, both provisions are the same, with the same obligations that the clinic has not fulfilled.

SIXTH.- Regarding the obligation to deliver documentation, both before the College of Dentists of Seville and in judicial proceedings and before this Agency, the clinic is obliged to deliver all documentation in its possession that forms part of the patient's clinical history. Moreover, the clinic is obliged to provide a copy of said clinical history to the patient when requested, exercising their right of access.

SIXTH: After granting a hearing to the summoned party, it states, in summary, that:

“The response given by A.A.A. to the allegations we previously made only serves to confirm what was maintained therein, namely, that all documentation generated by the treatment followed in our dental clinic is in their possession.

A different issue is that A.A.A. disagrees with the content of said documentation or believes that some document that was not generated is missing, but as we have already stated, this is a matter unrelated to the purpose of this Data Protection Agency and should be resolved, if appropriate, by the health or consumer authorities, which are competent to determine whether a clinical history or the declaration of conformity of a dental prosthesis meets the requirements demanded by health legislation.

The response from A.A.A. also denotes an absolutely twisted use of the procedure for exercising the right of access, as it openly acknowledges that its purpose is not the legally intended one of accessing the personal data that the clinic possesses or examining its treatment, but rather that its intention in claiming is to obtain documentation that is not the responsibility of the clinic.”

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clinic, as well as to prove that the necessary documents have not been generated and to demonstrate that errors have been made in the completion of others.

What is intended is, on one hand, to obtain documentation that, although it is not the clinic's responsibility to prepare, we understand it is the clinic's responsibility to ensure it reaches the patient in accordance with what the regulations require. On the other hand, to demonstrate to this agency that the documentation that cannot be provided is because throughout the treatment the clinic has not fulfilled its obligations to the patient, as stated in the current legislation” (bold and underlined text).

Or in the fourth allegation, by stating that:

“Regarding the informed consents, we do not understand what is absurd about requesting the informed consents corresponding to all treatments received at this clinic. The objective of this request is to demonstrate that the clinic has not provided the claimant with all the necessary informed consents, because if it had done so, they would be correctly signed” (bold and underlined text).

The claimant ultimately requests that her claim be admitted for processing, considering that the right of access is incomplete, but at no time does she specify which specific data subject to processing has not been made available to her, which shows that she resorts to the right of access as a quick way to obtain information that she intends to use in other areas, forgetting that the right of access has limits and should not be exercised for reasons unrelated to the protection of personal data, which A.A.A. at no time claims has been compromised.”

LEGAL GROUNDS

I

Competence

In accordance with the powers granted to each supervisory authority by Article 58.2 of Regulation (EU) 2016/679 (General Data Protection Regulation, hereinafter GDPR), and as established in Articles 47, 48.1, and 64.1 of the LOPDGDD, the Presidency of the Spanish Agency for Data Protection is competent to resolve this procedure.

Furthermore, Article 63.2 of the LOPDGDD states that: “The procedures processed by the Spanish Agency for Data Protection shall be governed by the provisions of Regulation (EU) 2016/679, by this

organic law, by the regulatory provisions issued for its development, and, in so far as they do not contradict them, subsidiarily by the general rules on administrative procedures.”

II

Preliminary Issues

In accordance with the provisions of Article 55 of the GDPR, the Spanish Agency for Data Protection is competent to perform the functions assigned to it in Article 57, including the enforcement of the Regulation and promoting awareness among data controllers and processors regarding their obligations, as well as addressing complaints submitted by a data subject and investigating, as appropriate, the reasons for them.

Correspondingly, Article 31 of the GDPR establishes the obligation of data controllers and processors to cooperate with the supervisory authority that requests it in the performance of its functions. In cases where they have designated a data protection officer, Article 39 of the GDPR assigns this officer the function of cooperating with that authority.

The opening of this procedure occurs due to the lack of attention to a request for the exercise of the rights established in Articles 15 to 22 of the GDPR, regulated in Article 64.1 of the LOPDGDD, which states:

“When the procedure refers exclusively to the lack of attention to a request for the exercise of the rights established in Articles 15 to 22 of Regulation (EU) 2016/679 of the European Parliament and of the Council, of April 27, 2016, it shall be initiated by an admission agreement, which shall be adopted in accordance with the provisions of Article 65 of this organic law.

In this case, the deadline for resolving the procedure shall be six months from the date on which the claimant was notified of the admission agreement. Once this period has elapsed, the interested party may consider their claim accepted.”

Article 58.2 of the GDPR grants the Spanish Agency for Data Protection a series of corrective powers to correct any infringement of the GDPR, including ordering the data controller or processor to address the requests for the exercise of the rights of the data subject under this Regulation.

III

Rights of Individuals Regarding Personal Data Protection

The rights of individuals regarding personal data protection are regulated in Articles 15 to 22 of the GDPR and Articles 13 to 18 of the LOPDGDD. These include the rights of access, rectification, erasure, objection, the right to restriction of processing, and the right to data portability.

The formal aspects related to the exercise of these rights are established in Articles 12 of the GDPR and 12 of the LOPDGDD.

Additionally, consideration is given to what is expressed in Recitals 59 and following of the GDPR.

In accordance with the provisions of these regulations, the data controller must establish formulas and mechanisms to facilitate the exercise of rights by the data subject, which shall be free of charge (without prejudice to the provisions of Articles 12.5 and 15.3 of the GDPR), and is obliged to respond to requests made no later than a

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unless it can demonstrate that it is unable to identify the interested party, and to express its reasons in case it does not intend to comply with such request. The burden of proof regarding the fulfillment of the duty to respond to the request for the exercise of rights made by the affected party lies with the responsible party.

The communication addressed to the interested party regarding their request must be expressed in a concise, transparent, intelligible, and easily accessible manner, using clear and simple language.

IV

Right of Access

In accordance with the provisions of Article 15 of the GDPR and Article 13 of the LOPDGDD, “the interested party has the right to obtain from the data controller confirmation as to whether or not personal data concerning them is being processed and, if so, the right of access to the personal data.” Regarding the right of access to personal data, according to the provisions of Article 13 of the LOPDGDD, when the exercise of the right refers to a large amount of data, the responsible party may

request the affected party to specify the "data or processing activities to which the request refers." The right will be considered granted if the responsible party provides remote access to the data, thus fulfilling the request (although the interested party may request information regarding the aspects provided in Article 15 of the GDPR).

The exercise of this right may be considered repetitive if it is exercised more than once within a six-month period, unless there is a legitimate reason for doing so. Furthermore, the request will be considered excessive when the affected party chooses a means other than the one offered that entails a disproportionate cost, which must be borne by the affected party.

Like the other rights of the interested party, the right of access is a personal right. It allows the citizen to obtain information about the processing of their data, the possibility of obtaining a copy of the personal data concerning them that is being processed, as well as information, in particular, about the purposes of the processing, the categories of personal data concerned, the recipients or categories of recipients to whom the data may be communicated, the expected retention period or criteria for retention, the possibility of exercising other rights, the right to lodge a complaint with the supervisory authority, the information available about the source of the data (if it has not been obtained directly from the data subject), the existence of automated decisions, including profiling, and information about transfers of personal data to a third country or to an international organization. The possibility of obtaining a copy of the personal data being processed will not adversely affect the rights and freedoms of others.

The right of access in relation to medical records is specifically regulated in Article 18 of Law 41/2002, of November 14, which is the basic regulatory law of the...

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Patient Autonomy and Rights and Obligations Regarding Information and Clinical Documentation (hereinafter, LAP), the literal text expresses:

"1. The patient has the right to access, with the reservations indicated in section 3 of this article, the documentation of the clinical history and to obtain a copy of the data contained therein. Healthcare centers will regulate the procedure that guarantees the observance of these rights.

2. The patient's right to access the clinical history may also be exercised by duly accredited representation.

3. The patient's right to access the documentation of the clinical history cannot be exercised to the detriment of the right of third parties to the confidentiality of the data contained therein collected in the therapeutic interest of the patient, nor to the detriment of the rights of the professionals involved in its preparation, who may oppose the right of access based on the confidentiality of their subjective notes.

4. Healthcare centers and individual practitioners will only provide access to the clinical history of deceased patients to individuals connected to them for family reasons or by fact, unless the deceased expressly prohibited it and this is evidenced. In any case, access by a third party to the clinical history motivated by a risk to their health will be limited to the relevant data. Information affecting the deceased's privacy or the subjective notes of professionals, or that harms third parties, will not be provided."

In this regard, it is important to highlight Article 15 of the LAP, which outlines the minimum content of the clinical history:

"1. The clinical history will incorporate the information deemed essential for the truthful and updated knowledge of the patient's health status. Every patient or user has the right to have a record, in writing or in the most appropriate technical support, of the information obtained in all their care processes, carried out by the health service both in primary care and specialized care.

2. The main purpose of the clinical history is to facilitate healthcare, documenting all those data that, under medical criteria, allow for the truthful and updated knowledge of the health status.

The minimum content of the clinical history will be as follows:

a) Documentation related to the clinical-statistical sheet.

- b) Admission authorization.
- c) Emergency report.
- d) Anamnesis and physical examination.
- e) Evolution.
- f) Medical orders.
- g) Consultation sheet.
- h) Reports of complementary examinations.
- i) Informed consent.
- j) Anesthesia report.
- k) Operating room or delivery record report.
- l) Pathological anatomy report.
- m) Nursing care evolution and planning.
- n) Therapeutic nursing application.
- ñ) Vital signs chart.
- o) Clinical discharge report.

Paragraphs b), c), i), j), k), l), ñ), and o) will only be required in the completion of the clinical history when it concerns hospitalization processes or as stipulated.

3. The completion of the clinical history, in aspects related to direct patient care, will be the responsibility of the professionals involved in it.

4. The clinical history will be maintained with criteria of unity and integration, in each healthcare institution at a minimum, to facilitate the best and most timely knowledge by the professionals of the data of a particular patient in each care process."

Regarding the preservation of the clinical history, Article 17 of the LAP, in its points 1 and 5, states:

"1. Healthcare centers have the obligation to preserve clinical documentation under conditions that guarantee its proper maintenance and security, although not necessarily in the original support, for the proper assistance to the patient for the appropriate time for each case and, at a minimum, five years from the date of discharge from each care process (...)

5. Healthcare professionals who carry out their activity individually are responsible for the management and custody of the care documentation they generate."

V

Conclusion

The rights procedure is initiated as a consequence of the lack of attention to any of the rights regulated in the data protection regulations. Therefore, in this case, only the facts related to the exercise of these rights will be analyzed and evaluated, excluding the rest of the issues raised by the parties.

It should be noted that this Agency is not competent to analyze and evaluate whether the documents that are part of the clinical history are or are not duly completed as established by the LAP previously mentioned, and the rest of the health regulations, allowing the claiming party to address the corresponding instances to raise their complaint on these aspects.

In this case, and considering the provisions in the previous paragraphs of the Legal Foundations, after examining all the documentation already related in the Facts, it has been established that the claiming party on February 12,

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On the 18th of January 2024, the claimant requested access to their medical history from the respondent, which was not addressed within the timeframe established by the regulations. However, following the submission of the complaint, the respondent has provided evidence of the response given to the claimant. In this regard, it is important to remember that it is not within the competence of this authority to assess whether the documents that make up the medical history sent by the respondent comply with the law, but rather to evaluate whether a reasoned response to the request was provided within the indicated timeframe and whether the claimant was given the information that,

as of the date of the access request, is contained in the medical history.

Nevertheless, the aforementioned regulations do not allow for the request to be overlooked as if it had not been made, leaving it without the response that those responsible are obligated to issue, even in cases where there are no data of the interested party in the entity's files or even in those cases where the requirements are not met, in which case the recipient of the request is also obliged to require the rectification of the observed deficiencies or, if applicable, to deny the request with justification indicating the reasons why the right in question cannot be considered.

Therefore, the request made obliges the responsible party to provide an express response, in any case, using any means that justifies the receipt of the response.

In this case, the respondent has provided documentation proving the communication sent to the interested party addressing the right or informing about the decision made regarding the request. This response, therefore, occurs after the established deadline has passed.

Consequently, considering that the purpose of this procedure is to ensure that the guarantees and rights of the affected parties are duly restored, combining the information in the file with the regulations referred to in the preceding sections, it is appropriate to uphold the present complaint on formal grounds due to the late issuance of the response, without requiring additional actions by the data controller to issue a new certification regarding the attention to the right.

In view of the cited provisions and other generally applicable regulations, the Presidency of the Spanish Agency for Data Protection RESOLVES:

FIRST: TO UPHOLD on formal grounds, the complaint filed by A.A.A. against CLÍNICA ANABEL REPULLO, S.L. However, it is not appropriate to issue a new certification by the respondent, as the response was issued late, and no additional actions are required from the responsible party.

SECOND: TO NOTIFY this resolution to A.A.A. and to CLÍNICA ANABEL REPULLO, S.L.

In accordance with the provisions of Article 50 of the LOPDGDD, this Resolution will be made public once it has been notified to the interested parties.

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Against this resolution, which concludes the administrative procedure in accordance with Article 48.6 of the LOPDGDD, and in accordance with the provisions of Article 123 of the LPACAP, the interested parties may optionally file a motion for reconsideration before the Presidency of the Spanish Agency for Data Protection within one month from the day following the notification of this resolution or directly file an administrative contentious appeal before the Contentious-Administrative Chamber of the National Court, in accordance with the provisions of Article 25 and Section 5 of the fourth additional provision of Law 29/1998, of July 13, regulating the Contentious-Administrative Jurisdiction, within two months from the day following the notification of this act, as provided in Article 46.1 of the aforementioned Law.

1381-090823

Lorenzo Cotino Hueso
President of the Spanish Agency for Data Protection
C/ Jorge Juan, 6
www.aepd.es
28001 – Madrid
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